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THE RELAPSE DYNAMIC

Relapse does not begin with the first drink. Relapse begins in a behavioral dynamic which reactivates patterns of denial, isolation, elevated stress, and impaired judgement. The pattern of this behavioral setup was identified in 1973 by the author through the completion of clinical interviews with 118 alcoholic patients who met the following criteria: (1) They had completed a 21 or 28 day intermediate care treatment program. (2) They had been discharged with the conscious intention to remain permanently sober. (3) They had eventually returned to loss of control consumption in spite of initial commitments to remain sober.

The results of this clinical research was compiled in the form of a Relapse Chart depicting the symptoms of a relapse. The most commonly reported symptoms are:

1. Apprehension About Well-Being — The alcoholic reported an initial sense of fear and uncertainty. There was a lack of confidence in the ability to stay sober. This apprehension was often extremely short lived.

2. Denial — The patient reactivated his denial system in order to cope with apprehension and resultant anxiety and stress. The denial systems reactivated in this stage of relapse dynamic tend to correspond with the denial systems utilized to deny the presence of alcoholism during the initial phase of treatment. Most patients were aware of this denial with hindsight but reported they were unaware of this denial process while experiencing it.

3. Adamant Commitment to Sobriety — The patient convinced himself he would "never drink again." This self persuasion was sometimes overt and blatant, but most often it constituted a very private decision. Many patients reported fear or apprehension of sharing that conviction with their therapist or with members of AA. Once a patient convinced himself he "would never drink again" the urgency of pursuing a daily program of recovery diminished.

4. Compulsive Attempts to Impose Sobriety on Others — This attempt to impose sobriety or individual standards for recovery on others was seldom overt. It was generally private judgements about the drinking of friends and spouses and the quality of the sobriety programs of fellow recovering alcoholics. When dealing with issues of sobriety, the patient began to focus more on what other persons were doing rather than on what he himself was doing.

5. Defensiveness — The patient reported a noticeable increase in his defensiveness when talking about his problems or recovery programs.

6. Compulsive Behavior — Behavior patterns became rigid and repetitive. The alcoholic tended to control conversational involvement either through monopoly or silence. The tendency toward overwork and compulsive involvement in activities began to appear. Nonstructured involvement with people was avoided.

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7. Impulsive Behavior — Patterns of compulsive behaviors began to be interrupted by impulsive reactions. In many cases the impulse was an overreaction to acute episodes of stress. There were also reports of impulsive activities being the culmination of a chronic stress situation. Many times these overreactions to stress formed the basis of decisions which affected major life areas and commitments to ongoing treatment.
8. Tendencies Toward Loneliness — Patterns of isolation and avoidance increased. There were generally valid reasons and excuses for this isolation. Patients reported short episodes of intense loneliness at increasing intervals. These episodes were generally dealt with by reactivating compulsive or impulsive behavior patterns rather than by pursuing responsible involvement with other persons.
9. Tunnel Vision — Patients tended to view their life in isolated fragments. They would focus exclusively on one area, pre-occupy themselves with it, and avoid looking at other areas. Sometimes pre-occupation was with the positive aspects thus creating a delusion of security and well-being. Others pre-occupied themselves with the negative aspects thus assuming a victim position which confirmed their belief they were helpless and being treated unfairly.
10. Minor Depression — Symptoms of depression began to appear and persist. Listlessness, flat acceptance, and oversleeping became common.
11. Loss of Constructive Planning — The patient's skills at life planning began to diminish. Attention to detail subsided. Wishful thinking began to replace realistic planning.
12. Plans Begin to Fail — Due to lack of attention to detail, or the pursuit of unrealistic objectives, the plans began to fail.
13. Idle Daydreaming and Wishful Thinking — The ability to concentrate diminished and concentration was replaced with fantasy. The "If Only Syndrome" became more common in conversation. The fantasies were generally of escape or of "being rescued from it all" by some unlikely set of circumstances.
14. Feelings That Nothing Can be Solved — A failure pattern in sobriety was developed. In some cases the failure was real in terms of objective realities, in other cases it was imagined and based upon intangibles. The generalized perception of "I've tried my best and it isn't working out," began to develop.
15. Immature Wish to be Happy — Conversational content and thought patterns became vague and generalized. The desire to "be happy" or "have things work out" became more common without ever defining what was necessary to be happy or have things work out.
16. Periods of Confusion — The episodes of confusion increased in terms of frequency, duration and severity of behavioral impairment.
17. Irritation with Friends — Social involvements including friends and intimate relationships, as well as treatment relationships formed with therapists and AA members, became strained and conflictual. The conflictual nature increased as confrontation of the alcoholic's progressively degenerating behavior increased.
18. Easily Angered — Episodes of anger, frustration, resentment and irritability increased. Overreaction became more frequent. Often the fear

of extreme overreaction to the point of violence seriously increased the level of stress and anxiety.

19. Irregular Eating Habits — The patient began overeating or undereating. The regular structure of meals was disrupted. Well-balanced meals were often replaced by less nourishing "junk foods."

20. Listlessness — Extended periods of inability to initiate action developed. These were marked by inability to concentrate, anxiety, and severe feelings of apprehension. Patients often reported this as a feeling of being trapped or of having no way out.

21. Irregular Sleeping Habits — Episodes of insomnia were reported. Nights of restlessness and fitful sleeping were reported. Episodes of sleeping marathons of 12-20 hours were reported at intervals varying between 6 and 15 days. These sleeping marathons apparently resulted from exhaustion.

22. Progressive Loss of Daily Structure — Daily routines became haphazard. Regular hours of retiring and rising disappeared. Inability to sleep resulted in oversleeping. Meal structures disappeared. Complaints of inability to keep appointments became more common, and social planning decreased. Patients reported feeling rushed and overburdened at times and then faced large blocks of idle time in which they didn't know what to do. An inability to follow through on plans and decisions was also reported. The patients reported they knew what they should do, but were unable to overcome strong feelings of tension, frustration, fear or anxiety that prevented them from following through.

23. Periods of Deep Depression — Depression became more severe, more frequent, more disruptive and longer in duration. These periods generally occurred during non-structured time periods and were amplified by fatigue and hunger. During these periods the patient tended toward isolation and reacted to human contact with irritability and anger while at the same time complaining that nobody cared.

24. Irregular Attendance at Treatment Meetings — Attendance at AA became sporadic. Therapy appointments were scheduled and then missed. Attendance at treatment groups and home AA meetings became sporadic. Rationalization patterns developed to justify this. The effectiveness of AA and treatment was discounted. Treatment lost a priority ranking in the patient value system.

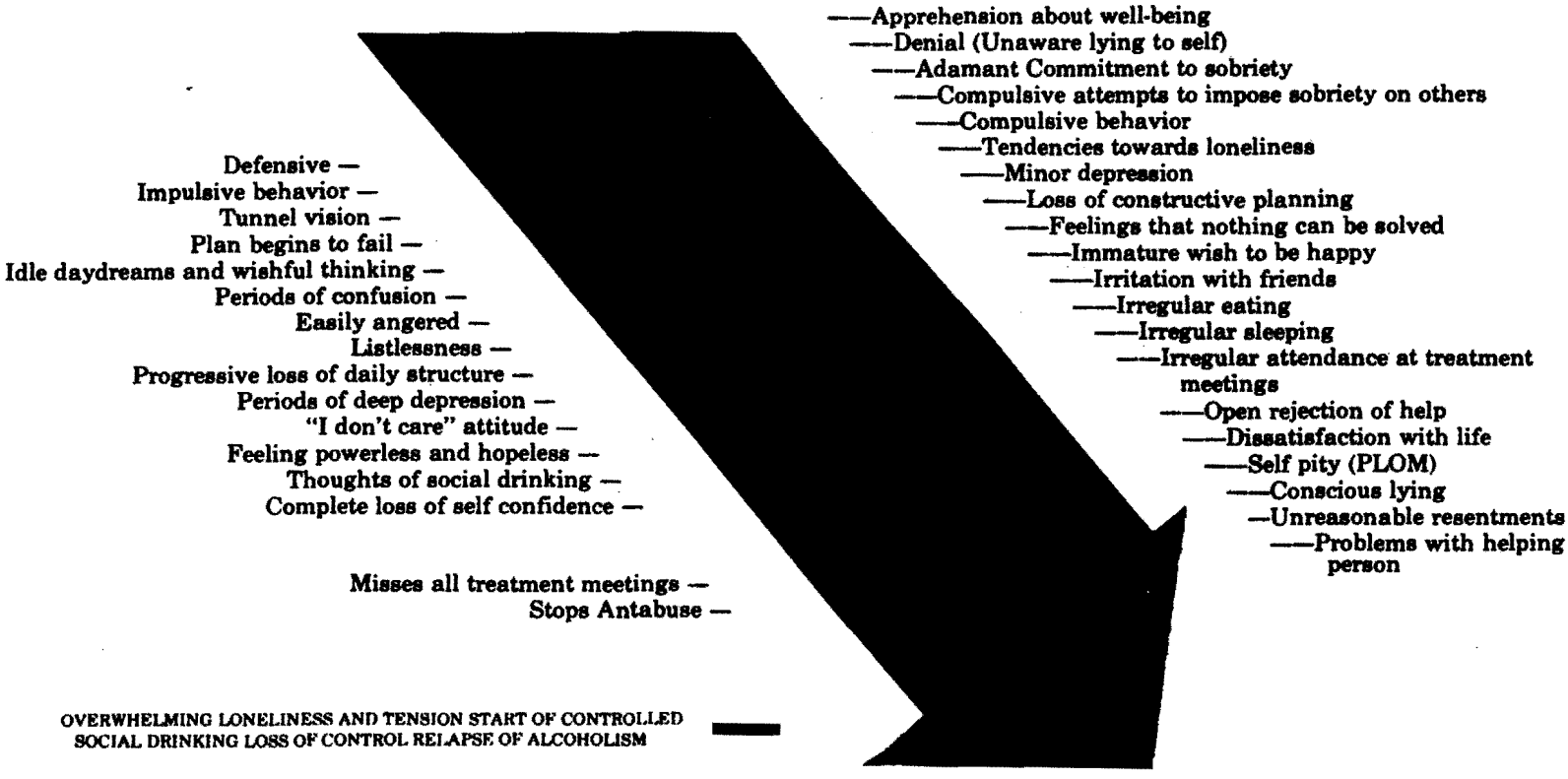
25. Development of an "I don't care" attitude — The patient generally reported this "I don't care" stance masked a feeling of helplessness and extremely poor self image.

26. Open Rejection of Help — The patient cut himself off from viable sources of help. This was sometimes accomplished dramatically through fits of anger or open discounts. Other times it was done through quiet withdrawal.

27. Dissatisfaction with Life — The patient began to think "things are so bad now I might as well get drunk because they can't get any worse." Rationalizations, tunnel vision and wishful thinking began to give way to the harsh reality of how totally unmanageable life had become in the course of this period of abstinance.

28. Feelings of Powerlessness and Helplessness — This was marked by an inability to initiate action. Thought processes were scattered, judgement was distorted, concentration and abstract thinking abilities were impaired.

29. Self Pity — The patient became indulgent in self pity. This is often called the PLOM (Poor Little Old Me) Syndrome. This self pity often was used as an attention getting device at AA and with family members.
30. Thoughts of Social Drinking — The patient realized that drinking could normalize many of the feelings and emotions he was experiencing. The hope that perhaps he could again drink in a controlled fashion began to emerge. Sometimes the thought was challenged and put out of conscious thought, other times it was entertained. Again, with hindsight, the patient realized he had few other alternatives but drinking. He felt he was facing a choice between insanity, suicide or a return to drinking.
31. Conscious Lying — Denial and rationalization became such extreme processes that even the alcoholic began to recognize the lies and deceptions. In spite of this recognition, he felt unable to interrupt the pattern.
32. Complete Loss of Self Confidence — The patient felt he couldn't get out of this trap no matter how hard he tried. He became overwhelmed by his inability to think clearly or initiate action.
33. Unreasonable Resentments — The Patient felt severe anger with the world in general and his inability to function. This anger was sometimes generalized; at other times focused at particular scapegoats; at other times turned against himself.
34. Discontinues All Treatment — Attendance at AA stops completely. Patients who were taking Antabuse report episodes of forgetting to take it or manipulations to avoid taking it regularly. When a helping person relationship was part of the treatment, strain and eventual termination of that relationship resulted. Patients dropped out of professional treatment in spite of a realization that they were acting irrationally and needed help.
35. Overwhelming Loneliness, Frustration, Anger and Tension — The patient reported feeling totally overwhelmed and feeling there were no available options except returning to drinking, suicide or insanity. The fear of insanity was intense. There was also intense feelings of helplessness and desperation. Often drinking was an impulsive behavior with little or no conscious preplanning.
36. Start Controlled Drinking — The efforts at control took two general patterns: the effort to control quantities while drinking on a regular basis, and the effort tc engage in one short-term and low consequence binge.
37. Loss of Control — The ability to control was lost, sometimes very quickly, sometimes after varying patterns of "controlled drinking." The patient, however, quickly returned to alcoholic drinking which was marked by symptoms as severe or more severe than were present during his last episode of active alcoholism.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Irregular sleeping
Irregular attendance at treatment meetings
Open rejection of help
Dissatisfaction with life
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Unreasonable resentments
Problems with helping person
Defensive
Impulsive behavior
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Plan begins to fail
Idle daydreams and wishful thinking
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Easily angered
Listlessness
Progressive loss of daily structure
Periods of deep depression
"I don't care" attitude
Feeling powerless and hopeless
Thoughts of social drinking
Complete loss of self confidence
Misses all treatment meetings
Stops Antabuse
OVERWHELMING LONELINESS AND TENSION START OF CONTROLLED SOCIAL DRINKING LOSS OF CONTROL RELAPSE OF ALCOHOLISM

For further information about training and consultation services based upon the CENAPS model of treatment contact:

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